

LABORATORY REPORT
Riverside Medical Center – Lab Services

PATIENT INFORMATION

Name: James Okafor
Date of Birth: 06/10/1975
Age: 51
Sex: Male
MRN: RMC-2026-19374
Ordering Physician: Dr. Helen Park, MD – Emergency Medicine
Collection Date: 08/09/2026 03:15 AM
Report Date: 08/09/2026 04:42 AM

CLINICAL INDICATION

Generalized weakness, confusion, and palpitations. Rule out electrolyte abnormality. Patient presented to ED via EMS after spouse found him disoriented at home.

RESULTS

COMPREHENSIVE METABOLIC PANEL (CMP)

Test	Result	Reference Range	Flag
Sodium	129 mEq/L	136–145 mEq/L	LOW ↓
Potassium	6.8 mEq/L	3.5–5.0 mEq/L	CRITICAL HIGH ↑↑
Chloride	98 mEq/L	98–106 mEq/L	Normal
CO2 (Bicarbonate)	18 mEq/L	23–29 mEq/L	LOW ↓
BUN	48 mg/dL	7–20 mg/dL	HIGH ↑
Creatinine	3.2 mg/dL	0.7–1.3 mg/dL	HIGH ↑
eGFR	19 mL/min	>60 mL/min	CRITICAL LOW ↓↓
Glucose	142 mg/dL	70–100 mg/dL	HIGH ↑
Calcium	8.1 mg/dL	8.5–10.5 mg/dL	LOW ↓
Total Protein	6.8 g/dL	6.0–8.3 g/dL	Normal
Albumin	3.2 g/dL	3.5–5.5 g/dL	LOW ↓
Total Bilirubin	0.9 mg/dL	0.1–1.2 mg/dL	Normal
Alkaline Phosphatase	88 U/L	44–147 U/L	Normal
AST	34 U/L	10–40 U/L	Normal
ALT	28 U/L	7–56 U/L	Normal

COMPLETE BLOOD COUNT (CBC)

Test	Result	Reference Range	Flag
WBC	11,800/μL	4,500–11,000/μL	HIGH ↑
RBC	3.21 M/μL	4.35–5.65 M/μL	LOW ↓
Hemoglobin	8.9 g/dL	13.5–17.5 g/dL	LOW ↓
Hematocrit	27.1%	38.3–48.6%	LOW ↓
MCV	84.4 fL	80–100 fL	Normal
Platelet Count	198 K/μL	150–400 K/μL	Normal

ADDITIONAL TESTS

Test	Result	Reference Range	Flag
Troponin I	0.02 ng/mL	<0.04 ng/mL	Normal
BNP	890 pg/mL	<100 pg/mL	HIGH ↑
Phosphorus	5.8 mg/dL	2.5–4.5 mg/dL	HIGH ↑
Magnesium	1.6 mEq/L	1.7–2.2 mEq/L	LOW ↓

URINALYSIS

Protein: 2+ (abnormal)
 Blood: 1+ (abnormal)
 Glucose: Trace
 Specific Gravity: 1.018

CRITICAL VALUE NOTIFICATION

** CRITICAL: Potassium 6.8 mEq/L – physician notified at 04:15 AM **
 ** CRITICAL: eGFR 19 mL/min – consistent with Stage 4 CKD **

Dr. Park notified of critical values via direct phone call at 04:15 AM on 08/09/2026. Read-back confirmed.

PATHOLOGIST COMMENT

Lab findings are consistent with acute kidney injury superimposed on chronic kidney disease. Severely elevated potassium requires immediate intervention. Anemia is normocytic, likely anemia of chronic disease. Elevated BNP suggests volume overload or cardiac stress. Recommend nephrology consultation and repeat electrolytes after treatment.

Reviewed and released by: Dr. Michael Torres, MD – Laboratory Medicine
 Date/Time: 08/09/2026 04:42 AM EST